ICU, Operating Theatre and Outpatient Department Operations

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1. Purpose

This document describes how the ICU, operating theatres and Outpatient Department are run at CareOps General Hospital from an operations point of view.

2. ICU admission and discharge flow

The ICU has 40 beds. Admission is decided by the ICU consultant based on clinical need and bed availability. When more patients need ICU than there are beds, the consultant prioritises those who are most likely to benefit and who cannot be safely managed elsewhere. ICU patients who become stable are moved to a step-down bed or ward as soon as the consultant agrees, so that the ICU bed is free for the next patient. The target is that a transfer-out order is carried out within 4 hours. ICU nursing ratio is 1 nurse for 2 patients.

3. ICU capacity alerts

ICU occupancy of 80 to 84 percent is Yellow, 85 to 94 percent is Orange, and 95 percent or more is Red. At Orange the ICU consultant reviews all patients for step-down. At Red the Duty Manager and the Medical Superintendent are informed, elective surgery that needs a postoperative ICU bed is postponed, and critical patients may be held in the ED or recovery under ICU-level care until a bed opens.

4. Operating theatre scheduling

The hospital has 7 scheduled theatres and 1 emergency theatre. Scheduled theatres open at 08:00 and the first case must start by 08:15. The target theatre utilization is 75 percent. The target turnover time between cases is 30 minutes. The operating list for the next day is finalised by 16:00 on the previous day, and the final list is shared with the ICU, anaesthesia, the Central Sterile Supply Department and the blood bank. Emergency cases take priority over elective cases and use the emergency theatre first.

5. Surgical safety checklist

The World Health Organization Surgical Safety Checklist is used for every operation. It has three phases. Sign In is done before induction of anaesthesia. Time Out is done before the skin incision, when the team confirms the patient, the site and the procedure. Sign Out is done before the patient leaves the operating room, when the team confirms the procedure recorded, the instrument and swab counts, and the specimen labelling.

6. Surgery cancellations

Common reasons for cancellation are no bed available after surgery, an incomplete pre-operative assessment, patient not fasting, equipment not ready and an emergency case taking the theatre. Every cancellation is recorded with its reason. A cancellation rate above 5 percent triggers a review. At Level 3 of the Overcrowding and Surge Capacity Plan, all non-urgent elective surgery is postponed.

7. Outpatient department operations

The OPD works from 08:00 to 20:00, Monday to Saturday, with 24 consultation rooms. Appointment slots are 15 minutes. The system allows overbooking of up to 10 percent of slots to compensate for no-shows. Registration should take no more than 10 minutes and the time from registration to consultation should be no more than 30 minutes. Appointment reminders are sent by message 24 hours before the visit. Patients who do not attend are recorded as no-shows. The usual no-show rate is 10 to 15 percent, and a rate above 20 percent for a clinic triggers a review of the reminder process.

8. OPD peaks and interaction with the ED

OPD demand is highest from 10:00 to 12:00. Long OPD waits push some patients to the ED for non-urgent problems, which increases ESI 4 and 5 arrivals. Follow-up appointments are therefore booked at the time of discharge. Teleconsultation slots are kept for follow-up visits that do not need examination.

9. Waiting list and prioritisation

Elective admissions are scheduled with priority for clinical urgency first and waiting time second. The admissions office reviews the elective list each morning against the bed forecast of the Bed Manager. If the forecast occupancy for the next day is 90 percent or more, elective admissions are reviewed for postponement.